PO No :PO10001145830-901

Customer Name	: Mr.SHASHANK DUBEY	Collected Via	: TATA 1MG HYDERABAD
Age/Gender	: 23/Male	Referred By	: Dr.
Lab Visit ID	: HYD734834	Collection Date	: 30/Jan/2026 09:14AM
Barcode ID/Order ID	: D23626410 / 15633112	Report Date	: 30/Jan/2026 06:33PM
Sample Type	: WHOLE BLOOD-EDTA	Report Status	: Final Report

HAEMATOLOGY

TCS WELLNESS PACKAGE PATHOLOGY CAMP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Complete Blood Count				
Hemoglobin	**17.3**	g/dL	13.0-17.0	Spectrophotometry (Cyanide-free)
RBC	**5.63**	10^6/cu.mm	4.5 - 5.5	Impedance
HCT	49.5	%	40 - 50	Calculated
MCV	87.9	fL	83 - 101	Calculated
MCH	30.7	pg	27 - 32	Calculated
MCHC	**34.9**	g/dL	31.5 - 34.5	Calculated
RDW-CV	13.8	%	11.5-14	Calculated
Total Leucocyte Count	6.62	10^3/µL	4 - 10	Impedance
Differential Leucocyte Count				
Neutrophils	**39.3**	%	40-80	DHSS/Microscopy
Lymphocytes	**49.7**	%	20-40	DHSS/Microscopy
Monocytes	7.7	%	2-10	DHSS/Microscopy
Eosinophils	2.9	%	1-6	DHSS/Microscopy
Basophils	0.4	%	0-2	Impedance/Microscopy
Absolute Leucocyte Count				
Absolute Neutrophil Count	2.6	10^3/µL	2 - 7	Calculated
Absolute Lymphocyte Count	**3.29**	10^3/µL	1-3	Calculated
Absolute Monocyte Count	0.51	10^3/µL	0.2 - 1	Calculated
Absolute Eosinophil Count	0.19	10^3/µL	0.02 - 0.5	Calculated
Absolute Basophil Count	0.03	10^3/µL	0.02-0.1	Calculated
Platelet Count	388	10^3/µL	150-410	Impedance /Microscopy
MPV	7.6	fL	6.5 - 12	Calculated
PDW	11.8	fL	9 - 17	Calculated

Comment:

As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood.
DHSS : Double Hydrodynamic Sequential System Flowcytometry
Calculated parameters are either derived from Impedence measure, RBC pulse measurement, RBC/platelet histograms or formula derived.

NABL certificate and scope

This test has been performed at
TATA 1MG HYDERABAD
Address: SCB Door No. 3-14-011, 1st Floor, Patny Square, SP Road, Rasoolpura, Secunderabad, Telangana - 500003

Dr. K Madhuri
MBBS, MD (Pathology)
Consultant Pathologist
Reg. No: 80149

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PO No :PO10001145830-901

Customer Name	: Mr.SHASHANK DUBEY	Collected Via	: TATA 1MG HYDERABAD
Age/Gender	: 23/Male	Referred By	: Dr.
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Sample Type	: WHOLE BLOOD-EDTA	Report Status	: Final Report

HAEMATOLOGY

TCS WELLNESS PACKAGE PATHOLOGY CAMP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Erythrocyte Sedimentation Rate				
Erythrocyte Sedimentation Rate	2	mm/hr	0-10	Modified Westergren

Comment:

- ESR provides an index of progress of the disease and is widely used as an indicator of inflammation, infection, trauma, or malignant diseases. Changes are more significant than a single abnormal test
- It is specifically indicated to monitor the course or response to the treatment of diseases like rheumatoid arthritis, tuberculosis bacterial endocarditis ,acute rheumatic fever ,Hodgkins disease,temporal arthritis , and systemic lupus erythematosis; and to diagnose and monitor giant cell arteritis and polymyalgia rheumatica.
- An elevated ESR may also be associated with many other conditions, including autoimmune disease, anemia, infection,malignancy,pregnancy, multiple myeloma, menstruation, and hypothyroidism.
- Although a normal ESR cannot be taken to exclude the presence of organic disease, its rate is dependent on various physiologic and pathologic factors.
- The most important component influencing ESR is the composition of plasma. High level of C-Reactive Protein, fibrinogen, haptoglobin, alpha-1antitrypsin, ceruloplasmin and immunoglobulins causes the elevation of Erythrocyte Sedimentation Rate.
- Drugs that may cause increase ESR levels include: dextran, methyldopa, oral contraceptives, penicillamine, procainamide, theophylline, and Vitamin A. Drugs that may cause decrease levels include: aspirin, cortisone, and quinine

PO No :PO10001145830-901

Customer Name	: Mr.SHASHANK DUBEY	Collected Via	: TATA 1MG HYDERABAD
Age/Gender	: 23/Male	Referred By	: Dr.
Lab Visit ID	: HYD734834	Collection Date	: 30/Jan/2026 09:14AM
Barcode ID/Order ID	: D23626410 / 15633112	Report Date	: 30/Jan/2026 05:46PM
Sample Type	: WHOLE BLOOD-EDTA	Report Status	: Final Report

HAEMATOLOGY

TCS WELLNESS PACKAGE PATHOLOGY CAMP

Test Name	Result	Unit	Bio. Ref. Interval	Method
HbA1c (Glycosylated Hemoglobin)				
Glycosylated Hemoglobin (HbA1c)	5.1	%	4-5.6	HPLC (NGSP certified)
Estimated average glucose (eAG)	99.67	mg/dL		Calculated

Comment:

Interpretation: HbA1c%

≤5.6	Normal
5.7-6.4	At Risk For Diabetes
≥6.5	Diabetes

Adapted from American Diabetes Association.

Comments:
A 3 to 6 monthly monitoring is recommended in diabetics. People with diabetes should get the test done more often if their blood sugar stays too high or if their healthcare provider makes any change in the treatment plan. HbA1c concentration represent the integrated values for blood glucose over the preceding 8-12 weeks and is not affected by daily glucose fluctuation, exercise & recent food intake.
Please note, Glycemic goal should be individualized based on duration of diabetes, age/life expectancy, comorbid conditions, known CVD or advanced microvascular complications, hypoglycemia unawareness, and individual patient considerations.

Factors that interfere with HbA1c Measurement: Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements.

Factors that affect interpretation of HbA1c Measurement: Any condition that shortens erythrocyte survival or decrease mean erythrocyte age (e. g., recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used. Iron deficiency anemia is associated with higher HbA1c.

Note: Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1c result does not correlate with the patient's blood glucose levels.

- HPLC - High performance liquid chromatography

NABL certificate and scope

This test has been performed at
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PO No :PO10001145830-901

Customer Name	: Mr.SHASHANK DUBEY	Collected Via	: TATA 1MG HYDERABAD
Age/Gender	: 23/Male	Referred By	: Dr.
Lab Visit ID	: HYD734834	Collection Date	: 30/Jan/2026 09:14AM
Barcode ID/Order ID	: D23626407 / 15633112	Report Date	: 30/Jan/2026 05:34PM
Sample Type	: Fluoride Plasma F	Report Status	: Final Report

BIOCHEMISTRY

TCS WELLNESS PACKAGE PATHOLOGY CAMP

Test Name	Result	Unit	Bio. Ref. Interval	Method
FBS (Fasting Blood Sugar)				
Glucose - Fasting	86	mg/dL	70 - 99	Hexokinase

Fasting Plasma Glucose (mg/dL)	2 hr plasma Glucose (mg/dL)	Diagnosis
99 or below	139 or below	Normal
100 to 125	140 to 199	Pre-Diabetes (IGT)
126 or above	200 or above	Diabetes

Reference : American Diabetes Association

Comment:

Impaired glucose tolerance (IGT) fasting, means a person has an increased risk of developing type 2 diabetes but does not have it yet. A level of 126 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes.
IGT (2 hrs Post meal), means a person has an increased risk of developing type 2 diabetes but does not have it yet. A 2-hour glucose level of 200 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes

Plasma Glucose Goals	For people with Diabetes
Before meal	70-130 mg/dL
2 Hours after meal	Less than 180 mg/dL
HbA1c	Less than 7%

PO No :PO10001145830-901

Customer Name	: Mr.SHASHANK DUBEY	Collected Via	: TATA 1MG HYDERABAD
Age/Gender	: 23/Male	Referred By	: Dr.
Lab Visit ID	: HYD734834	Collection Date	: 30/Jan/2026 09:14AM
Barcode ID/Order ID	: D23626408 / 15633112	Report Date	: 30/Jan/2026 10:47PM
Sample Type	: Serum	Report Status	: Final Report

BIOCHEMISTRY

TCS WELLNESS PACKAGE PATHOLOGY CAMP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Lipid Profile				
Cholesterol - Total	151	mg/dL	Low (desirable): < 200 mg/dL Moderate (borderline) 200–239 mg/dL High: >/= 240 mg/dL	CHOD-POD
Triglycerides	78	mg/dL	Normal: <150, Borderline: 150 - 199, High:200-499, Very High>=500	GPO
Cholesterol - HDL	47	mg/dL	High risk <40mg/dL Low risk >=40mg/dl	Accelerator Selective Detergent
Cholesterol - LDL	88	mg/dl	Desirable: <100 Above desirable: 100 - 129 Borderline high : 130 - 159 High : 160 - 189 Very high : >=190	Calculated
Cholesterol- VLDL	16	mg/dl	<30	Calculated
Cholesterol : HDL Cholesterol	3.2	Ratio	Desirable : 3.5-4.5 High Risk : >5	Calculated
LDL : HDL Cholesterol	1.87	Ratio	Desirable : 2.5-3.0 High risk : >3.5	Calculated
Non HDL Cholesterol	104	mg/dL	Desirable:< 130, Above Desirable:130 - 159, Borderline High:160 - 189, High:190 - 219, Very High: >= 220	Calculated

Comment:

NABL certificate and scope

This test has been performed at
TATA 1MG HYDERABAD
Address: SCB Door No. 3-14-011, 1st Floor, Patny Square, SP Road, Rasoolpura, Secunderabad, Telangana - 500003

Dr. G Munni Krishna
MD Biochemist
APMC / FMR/84082

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PO No :PO10001145830-901

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Lab Visit ID	: HYD734834	Collection Date	: 30/Jan/2026 09:14AM
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BIOCHEMISTRY

TCS WELLNESS PACKAGE PATHOLOGY CAMP

Test Name	Result	Unit	Bio. Ref. Interval	Method

●Lipid results show analytical and biological variation; repeat testing may be recommended before diagnosis or treatment decisions.
●Indians lie at high risk of developing early (a decade earlier than western populations) and more severe cardiovascular disease (ASCVD); higher mortality. Dyslipidemia (abnormal lipid profile) affects nearly 80% of population.
●**Total cholesterol** is the sum of all cholesterol in the blood, including HDL, LDL, VLDL, and remnants.
●**LDL Cholesterol (LDL-C**), is the main "bad" cholesterol that contributes to plaque buildup, increasing the risk of heart disease and stroke, typically calculated by the Friedewald formula. Direct LDL-C measurement by homogeneous enzymatic assays carried out when triglycerides >400 mg/dL or dysbetalipoproteinemia.
●**High-density lipoprotein (HDL**) or "good" cholesterol is anti-atherogenic (protective). Low HDL-C is a cardiovascular risk factor; seen in almost two-third of Indians. Values above 60 mg/dl are considered protective.
●**Triglyceride (TG)** are a key driver of CVD. Indians are especially prone to atherogenic dyslipidemia—high TG, low HDL-C, and high LDL-C—closely linked to diabetes, metabolic syndrome, and insulin resistance; making TG management crucial.
●**Non-HDL-Cholesterol (non-HDL-C)** Non-HDL-C measures all plaque-forming lipoproteins and is vital to monitor in high-TG patients (e.g., diabetics, obese) and those on statin therapy.
●**Lipid Association of India (LAI-2020) recommends:-**

- Screening of all Indians above the age of 20 years for CVD risk factors, esp. lipid profile.
- Identification of Major Risk factors: Age (male ≥45 years, female ≥55 years); Family h/o heart disease at younger age (<55 yrs in males, <65 yrs in female or before menopause), current smoking/tobacco use, High blood pressure, Low HDL (males <40 mg/dl and females <50mg/dl).
- Fasting not mandatory; both fasting and non-fasting lipid profiles are useful for screening in Indian patients.
- LAI identifies both LDL-C and non-HDL-C as risk factors and recommends LDL-C, non-HDL-C and Apo-B as targets of lipid-lowering therapy.
- Lifestyle changes are the first-line approach for managing and preventing dyslipidemia. Treatment in low-risk individuals is initiated only after 3 months of unsuccessful lifestyle modification.
- Testing for Apolipoprotein B(Apo-B), hsCRP, Lp(a) should be considered for patients in moderate risk group.

Treatment targets for lipid-lowering therapy for various ASCVD risk groups

Risk Group	Treatment targets		
	LDL-C, mg/dL (primary target)	Non-HDL-C, mg/dL (co-primary target)	Apo-B, mg/dL (secondary target)
Low-risk group	<100	<130	<90
Moderate-risk group	<100 (optional <70)	<130 (optional <100)	<90
High-risk group	<70	<100	<80
Very high-risk group	<50	<80	<65
Extreme-risk group- category A	<50 (optional ≤30)	<80 (optional ≤60)	<65
Extreme-risk group- category B	≤30	≤60	<50
Extreme-risk group- category C	10-15	40-45	-

Source: LAI (2024) Consensus Statement IV

•Per **NCEP Expert Panel (2011) guidelines,** universal dyslipidemia screening is advised at 9–11 years and repeated at 17–21 years. Screening before age 2 yrs is not recommended; from age 2 onward, selective screening is done for children with a family history of premature CVD or risk factors such as obesity, diabetes, or hypertension.

NABL certificate and scope

This test has been performed at
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BIOCHEMISTRY

TCS WELLNESS PACKAGE PATHOLOGY CAMP

Note: Biological Reference Interval as per National Cholesterol Education Program (NCEP) ATP III and LAI guidelines

LIVER FUNCTION TEST

Liver Function Test

Test Name	Result	Unit	Bio. Ref. Interval	Method
Bilirubin-Total	**2.27**	mg/dL	0.3 – 1.2	Vanadate oxidation
Bilirubin-Direct	**0.91**	mg/dL	0.0-0.3	Vanadate oxidation
Bilirubin-Indirect	**1.36**	mg/dL	0.2-0.8	Calculated
Protein, Total	7.40	g/dL	5.7–8.2	Biuret
Albumin	4.74	g/dL	3.2-4.8	BCG Dye Binding
Globulin	2.7	g/dL	2.1 - 3.9	Calculated
A/G Ratio	1.78	Ratio	0.8 - 2.1	Calculated
SGOT (Aspartate Aminotransferase)	23	U/L	<34	Modified IFCC
SGPT (Alanine Transaminase)	44	U/L	10-49	Modified IFCC
SGOT/SGPT	0.52	Ratio		Calculated
Alkaline Phosphatase	75	U/L	46-116	IFCC Standardization
Gamma Glutamyltransferase (GGT)	17	U/L	<73	Modified IFCC

Comment:

- Raised ALT and AST indicate hepatocellular damage (e.g. viral or drugs etc). ALT is more liver-specific while AST is also found in heart, skeletal muscle, and kidney. Mild elevation (less than twice normal) often resolves on its own. Fatty liver disease (especially with metabolic syndrome) is a common cause in asymptomatic cases. Certain drugs (paracetamol, statins), herbal supplements, energy drinks, and antibiotics may also affect liver function.
- SGOT/SGPT Ratio: Typically <1 in healthy individuals (vary between 0.7-1.4; higher in women than men). High SGPT (ratio <1) seen in acute or chronic hepatitis, autoimmune disorders, medications, toxins while ratio >1 indicates alcoholic hepatitis, cirrhosis, metastasis or non-hepatic issues (hemolytic diseases, CVS disorders).
- Elevated Alkaline Phosphatase and GGT: Suggest cholestatic diseases (e.g. bile duct obstruction, primary biliary cirrhosis etc.) and can also be due to bone disease, pregnancy, chronic renal failure, malignancy, and congestive heart failure.
- High Bilirubin: Indicates jaundice due to increased RBC breakdown, liver damage (e.g., infections, toxins), or cholestasis (e.g., gallstones, tumors).
- High Protein Levels: Seen in dehydration (e.g., severe vomiting, diarrhea) or increased production (e.g., inflammation, hematopoietic neoplasms). Low protein and albumin: Result from impaired synthesis (liver disease), decreased intake,

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BIOCHEMISTRY

TCS WELLNESS PACKAGE PATHOLOGY CAMP

Test Name	Result	Unit	Bio. Ref. Interval	Method
tissue damage, malabsorption, or increased renal excretion.				
Kidney Function Test.				
Blood Urea Nitrogen	12	mg/dL	9.0 - 23.0	Urease with GLDH
Urea	25.68	mg/dL	19.26 - 49.22	Calculated
Creatinine	1.07	mg/dL	0.7-1.3	Alkaline picrate-kinetic
Uric Acid	5.9	mg/dL	3.5-7.2	Uricase/Peroxidase
Sodium	143	mEq/L	136-145	Indirect ISE
Potassium	4.35	mEq/L	3.5-5.1	Indirect ISE
Chloride	103.0	mmol/L	98-107	Indirect ISE
BUN/Creatinine Ratio	11.2	Ratio	12:1 - 20:1	Calculated

Comment:

BUN is directly related to protein intake and nitrogen metabolism and inversely related to the rate of excretion of urea.Blood urea nitrogen (BUN) levels reflect the balance between the production and excretion of urea. Increased levels are seen in renal failure (acute or chronic), urinary tract obstruction, dehydration, shock, burns, CHF, GI bleeding, nephrotoxic drugs. Decreased levels are seen in hepatic failure, nephrotic syndrome, cachexia (low-protein and high-carbohydrate diets).
Urea is a non-proteinous nitrogen compound formed in the liver from ammonia as an end product of protein metabolism. Urea diffuses freely into extracellular and intracellular fluid and is ultimately excreted by the kidneys. Increased levels are found in acute renal failure, chronic glomerulonephritis, congestive heart failure, decreased renal perfusion, diabetes, excessive protein ingestion, gastrointestinal (GI) bleeding, hyperalimentation, hypovolemia, ketoacidosis, muscle wasting from starvation, neoplasms, pyelonephritis, shock, urinary tract obstruction, nephrotoxic drugs. Decreased levels are seen in inadequate dietary protein, low-protein/high-carbohydrate diet, malabsorption syndromes, pregnancy, severe liver disease, certain drugs.
Creatinine is catabolic product of creatinine phosphate, which is excreted by filtration through the glomerulus and by tubular secretion. Creatinine clearance is an acceptable clinical measure of glomerular filtration rate (GFR). Increased levels are seen in acute/chronic renal failure, urinary tract obstruction, hypothyroidism, nephrotoxic drugs, shock, dehydration, congestive heart failure, diabetes. Decreased levels are found in muscular dystrophy.
BUN/Creatinine ratio (normally 12:1–20:1) is decreased in acute tubular necrosis, advanced liver disease, low protein intake, and following hemodialysis. BUN/Creatinine ratio is increased in dehydration, GI bleeding, and increased catabolism.
Uric acid levels show diurnal variation. The level is usually higher in the morning and lower in the evening. Increased levels are seen in starvation, strenuous exercise, malnutrition, or lead poisoning, gout, renal disorders, increased breakdown of body cells in some cancers (including leukemia, lymphoma, and multiple myeloma) or cancer treatments, hemolytic anemia, sickle cell anemia, or heart failure, pre-eclampsia, liver disease (cirrhosis), obesity, psoriasis, hypothyroidism, low blood levels of parathyroid hormone (PTH), certain drugs, foods that are very high in purines - such as organ meats, red meats, some seafood and beer. Decreased levels are seen in liver disease, Wilson's disease, Syndrome of inappropriate antidiuretic hormone (SIADH), certain drugs.

NABL certificate and scope

This test has been performed at
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Barcode ID/Order ID	: D23626408 / 15633112	Report Date	: 30/Jan/2026 05:36PM
Sample Type	: Serum	Report Status	: Final Report

IMMUNOLOGY

TCS WELLNESS PACKAGE PATHOLOGY CAMP

Test Name	Result	Unit	Bio. Ref. Interval	Method
Thyroid Stimulating Hormone, Ultrasensitive				
Thyroid Stimulating Hormone - Ultrasensitive	2.026	uIU/ml	0.55-4.78	CLIA

Comment:

Pregnancy	Reference ranges for TSH (µIU/ml) [As per American thyroid Association]
1st trimester	0.1-2.5
2nd trimester	0.2-3.0
3rd trimester	0.3-3.0

- TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm .
- The variation is of the order of 50%, hence time of the day has influence on the measured serum TSH concentrations.
- TSH is secreted in a dual fashion: Intermittent pulses constitute 60-70% of total amount, background continuous secretion is 30-40%.These pulses occur regularly every 1-3 hrs.
- TSH is a very sensitive and specific parameter for assessing thyroid function and is particularly suitable for early detection or exclusion of disorders in the central regulating circuit between the hypothalamus, pituitary and thyroid.
- Changes in thyroid status are typically associated with concordant changes in T3, T4 and TSH levels.
- For the diagnosis of hypothyroidism and hyperthyroidism, sole dependence on TSH should not be done and assay needs to be interpreted with the clinical condition & other investigations.
- Serum TSH level changes significantly in response to even minor changes in thyroid hormones.
- Transient increase in TSH level or an abnormal TSH levels can be seen in various nonthyroidal diseases.
- Unexpectedly abnormal or discordant thyroid test values may be seen with some rare, but clinically significant conditions such as central hypothyroidism, TSH-secreting pituitary tumors, thyroid hormone resistance, or the presence of heterophilic antibodies (HAMA) or thyroid hormone autoantibodies.

NABL certificate and scope

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